Tamoxifen · daily hormone therapy

Take tamoxifen once daily continuously. There is no planned week off. The prescriber confirms the duration for the treatment setting.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Read the row for your treatment setting. Columns show order, not equal lengths of time. Agree the due window, booking team, result reviewer and whom to contact if a check is missing.

Check	Before starting	First weeks	During treatment	Longer-term review	Change / completion
Bloods / symptom review	Blood count, kidney and liver profile; discuss calcium check	Review symptoms, medicines and product-specific blood plan	Irish Tamox: regular blood count, liver and calcium checks; agree frequency	Review adherence, symptoms and results	Follow unresolved abnormalities
Heart-rhythm checks	ECG and electrolytes if heart-rhythm risk indicates	Repeat according to risk, symptoms and product	Prompt review for palpitations, fainting or dizziness	No routine echo timetable for tamoxifen alone	Follow any identified abnormality
Bone density / DXA	Assess fracture risk and menopausal state	DXA only when indicated	No cycle-based DXA appointments	Repeat only according to the bone-risk plan	Continue bone care according to risk
Early-breast surveillance	Confirm early / curative setting	Set first follow-up mammogram date	Assess new symptoms promptly	Annual mammography of remaining breast tissue	No routine CT / markers if asymptomatic
Advanced-disease response	Document disease sites / baseline assessment	New symptoms can bring assessment forward	Generally every 2–4 months; individualise	Adapt interval and imaging to disease / benefit	Markers supplement, never replace, assessment
Gynaecological / eye review	Irish Tamox recommends an eye examination before treatment	Prompt assessment of new bleeding or visual symptoms	Eye review if symptoms; no routine screening ultrasound or biopsy	NCCP annual gynaecological examination	Follow symptoms and any abnormal findings

THE WHOLE TREATMENT JOURNEY

Tamoxifen · daily hormone therapy: treatment and checks

ER-positive early or advanced breast cancer

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Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First weeks	During treatment	Longer-term review	Change / completion
Treatment plan NCCP 00253	Confirm indication and prescription	Start the agreed daily / injection schedule	Continue unless prescriber changes the plan	Review duration, benefit and tolerability	Agree continuation, switch or completion
Bloods / symptom review NCCP 00253	Blood count, kidney and liver profile; discuss calcium check	Review symptoms, medicines and product-specific blood plan	Irish Tamox: regular blood count, liver and calcium checks; agree frequency	Review adherence, symptoms and results	Follow unresolved abnormalities
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Early: radiotherapy ESMO_EBC2024	Review surgery / pathology	Plan radiotherapy if indicated	Coordinate endocrine timing	Follow the early-breast plan	Review late effects when relevant

What each check is for

1 Confirm the plan

Before starting or changing treatment

Sources: NCCP 00253

2 Checks and symptoms

At the agreed review points

Sources: NCCP 00253

3 Review ongoing benefit

At setting-specific decision points

Sources: NCCP 00253

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

SACTCheck · Developed by Dr Paul O'Brien · v0.84.20 · Sources checked 2026-10-25
Source reconciled; independent clinical, pharmacy and patient/PPI review pending